SPECIALIST SCRIPTS · 25–35 SECONDS

16 Scripts, Rebuilt as a Specialist

Your scripts, recreated in your direction: specific, not generic; specialist, not creator. Every script is 25–35 seconds, with one strong message and value in every line.

Prepared for Dr. Azka · 5 November 2026 · Your 15 scripts + 1 new ('Knowledge, not time')

Overview

#	Script	Stage	Words	≈ Length
1	4 mistakes quietly costing doctors patients	TOFU	93	~36 s
2b-A	If I had to take a dental clinic from zero	MOFU	86	~33 s
2b-B	If I took over a dermatologist's Instagram from zero	MOFU	91	~35 s
3b	Doctors who feel awkward on camera	MOFU	71	~27 s
NEW	Doctors have the knowledge, not the time	MOFU	86	~33 s
4b	If I handled a dermatologist's Instagram, these are this week's 6 posts	TOFU	90	~35 s
5b	Your 3 AM inbox is someone's 11 PM in London	MOFU	81	~31 s
6b	The doctor vs. influencer test	BOFU	75	~29 s
7b	200K views and zero patients? Check this one screen.	TOFU	73	~28 s
Clinic 1	Multi-doctor clinic? Your page shouldn't only be about the owner	TOFU	90	~35 s
Clinic 2	The 4-line bio rebuild I do for every cosmetic dentist	BOFU	87	~33 s
Clinic 3	Day 1 of building a clinic brand: I don't film anything	MOFU	86	~33 s
4	Two dermatologists, same content: one booked for weeks	MOFU	74	~28 s
5	One personal message beats ten promotions	MOFU	71	~27 s
6	As a doctor, here's where I'd use AI in a clinic, and where I never would	TOFU	64	~25 s
7	The 7 systems behind a $30k+/month clinic	BOFU	79	~30 s

The rules these scripts follow (your direction)

- **Specialist, not creator.** Every point should make a doctor think "I've never heard that before." No advice they've heard 100 times.
- **Specific over generic.** Exact wording, exact screens to check, exact numbers to count, real patient questions.
- **25–35 seconds** (≈65–90 spoken words). Never over 50.
- **One core message** per script, the line doctors repeat to colleagues.
- **Easy CTAs:** one-word or one-number answers that start conversations with doctors.

> **Timing**
> Lengths are estimated at a natural ~2.6 words per second. Read each one aloud once. If you go over 35 seconds, cut a line; don't speed up.

SCRIPT 1 · TOFU · 93 WORDS · ~36 S

4 mistakes quietly costing doctors patients

Core message: The mistakes costing you patients aren't the obvious ones. They're the ones nobody talks about.
Why it's specialist, not generic: Replaces the usual 'no dancing, no jargon' advice with mistakes doctors haven't heard: copied structures, putting others down, and being invisible to ChatGPT.

TIME	SAY	ON SCREEN / VISUAL
0–3s	Four mistakes quietly costing you patients. No one talks about them.	OST: “4 mistakes no one talks about”
3–10s	One: copying another creator's script structure word for word. Patients have heard that exact video from ten other doctors.	OST: “1. Copied structures”
10–16s	Two: ‘this treatment is good, that one's bad’. Putting others down makes patients nervous, not confident.	OST: “2. Putting others down”
16–25s	Three: only thinking about Google. Patients now ask ChatGPT for the best dermatologist near them. If your content doesn't say what you treat and where, AI can't recommend you.	OST: “3. Invisible to ChatGPT”
25–32s	Four: results with no reasoning. The result proves you can. Your reasoning proves why they should choose you.	OST: “4. Results without reasoning”

CTA: Which one are you guilty of? Be honest: 1, 2, 3 or 4.

SCRIPT 2B-A · MOFU · 86 WORDS · ~33 S

If I had to take a dental clinic from zero

Core message: Content decides who reaches out. Your reply speed decides who books.
Why it's specialist, not generic: Exact name-field wording, price as a range, and a reply rule with two time slots. Things a dentist can do today.

TIME	SAY	ON SCREEN / VISUAL
0–3s	Dental clinic from zero to a full chair? Here's the exact order.	OST: “Dental clinic from zero”
3–9s	One: name field. ‘Dr. Name, Implant Dentist, Dubai Marina’. That's what patients type.	OST: “1. Searchable name”
9–14s	Two: one flagship treatment. Implants or veneers, not twelve services.	OST: “2. One flagship”
14–21s	Three: three videos. What it costs, as a range and what changes it. Does it hurt. How long it takes.	OST: “3. Cost · Pain · Time”
21–26s	Four: film the chair, the room, the team. Nervous patients book places they've seen.	OST: “4. Show the clinic”
26–33s	Five: reply within minutes, with two time slots. Content gets the inquiry. Your reply gets the booking.	OST: “5. Fast reply + 2 slots”

CTA: Send this to a dentist who's posting but not booking.

SCRIPT 2B-B · MOFU · 91 WORDS · ~35 S

If I took over a dermatologist's Instagram from zero

Core message: Start with your patients' questions, not other people's videos.
Why it's specialist, not generic: Your specialist process: topics (not videos) → real comments → pillars → niche structures, so the content is driven by the patient, not copied.

TIME	SAY	ON SCREEN / VISUAL
0–3s	If I took over a dermatologist's Instagram from zero, I wouldn't start by copying viral videos. Here's what I'd do.	OST: "Derm from zero: my order"
3–10s	First, I'd pick the 10 to 15 best topics. Topics, not videos. Melasma, acne scars, laser downtime.	OST: "1. Topics, not videos"
10–18s	Second, I'd read every comment, reply to all of them, and turn the questions into content. Your patients are writing your scripts for you.	OST: "2. Comments = content"
18–24s	Third, build the pillars from those topics and comments, not guesswork.	OST: "3. Pillars from real questions"
24–32s	Only then, study the script structures working in your niche, so you know the pattern, and do it differently.	OST: "4. Study structures. Then be different."

CTA: Dermatologists: what's the question patients ask you most? Drop it below.

SCRIPT 3B · MOFU · 71 WORDS · ~27 S

Doctors who feel awkward on camera

Core message: Don't present to an audience. Talk to one patient, like you do every day.
Why it's specialist, not generic: A doctor-specific reframe (consultation mode) and two tiny habits, in 30 seconds.

TIME	SAY	ON SCREEN / VISUAL
0–3s	If you feel awkward on camera as a doctor, I felt exactly the same.	OST: “Awkward on camera? Same.”
3–9s	We studied anatomy and pharmacology. Nobody taught us cameras.	Visual: you setting up your phone
9–19s	What fixed it: I stopped talking to ‘an audience’. I picture one patient across my desk, a conversation you've had hundreds of times.	OST: “Talk to ONE patient”
19–26s	Then two habits: one point per video, and never use take one.	OST: “One point · Never take one”
26–30s	You don't need to be a presenter. Just the doctor you already are.	OST: “Be the doctor you already are.”

CTA: Follow for more doctor-to-doctor filming tips.

SCRIPT NEW · MOFU · 86 WORDS · ~33 S

Doctors have the knowledge, not the time

Core message: It's not a skill problem. It's a time problem.
Why it's specialist, not generic: Built from real sales-call insight. Names the exact work doctors can't do (research, hooks, scripts) and positions you without hard selling.

> ⚠ **Note**
> Replace [X] with your real number of calls. Only a true number.

TIME	SAY	ON SCREEN / VISUAL
0–3s	This week I spoke to [X] doctors on sales calls. Almost all said the same thing.	OST: "[X] doctors. Same problem."
3–10s	They have years of knowledge, but don't know how to fit it into 30 seconds and keep people watching.	OST: "Knowledge ✔ · 30 seconds ✖"
10–20s	That's understandable. A practising doctor doesn't have 3–4 hours a day to research, find hooks and write scripts. Most barely have time to film.	OST: "No time for: research · hooks · scripts"
20–25s	So it's not a skill problem. It's a time problem.	OST: "Not skill. Time."
25–33s	Learn the system, or hand it over. That's what we do: your expertise, turned into content.	OST: "Your expertise → our system"

CTA: Which one are you: learn it, or hand it over? Tell me below.

SCRIPT 4B · TOFU · 90 WORDS · ~35 S

If I handled a dermatologist's Instagram, these are this week's 6 posts

Core message: Every post this week comes from real patients and your real cases, not trends.
Why it's specialist, not generic: Your specialist weekly plan: comment questions, storytelling transformations, the patient journey, case reasoning, your story, and niche structures.

> ⚠ **Note**
> Days 2–4: patient consent in writing, and 'results vary'.

TIME	SAY	ON SCREEN / VISUAL
0–3s	If I handled a dermatologist's Instagram, here are this week's six posts.	OST: “Derm: this week's 6 posts”
3–8s	Day one: answer a question people asked in your comments.	OST: “Day 1: A comment question”
8–14s	Day two: a transformation, told as a story. Where she started, what worried her, what changed.	OST: “Day 2: Transformation story”
14–19s	Day three: one patient's journey, from first consult to final result.	OST: “Day 3: Patient journey”
19–25s	Day four: a case study. How you thought it through, what you chose, and why.	OST: “Day 4: Your reasoning”
25–29s	Day five: your story, or a day in your life.	OST: “Day 5: You”
29–35s	Day six: a script structure that's working for top dermatologists in your speciality, done your way.	OST: “Day 6: Proven structure, your way”

CTA: Save this as your week. Which day would you find hardest to film?

SCRIPT 5B · MOFU · 81 WORDS · ~31 S

Your 3 AM inbox is someone's 11 PM in London

Core message: International patients choose a Dubai doctor online, and often message while you're asleep.
Why it's specialist, not generic: The time-zone angle is unique to Dubai's medical-tourism market, and it ties visibility to your AI reply systems. No unverified statistic needed.

TIME	SAY	ON SCREEN / VISUAL
0–3s	Dubai doctors: your 3 AM inbox is someone's 11 PM in London.	OST: "3 AM Dubai = 11 PM London"
3–12s	Patients flying in for treatment compare three or four Dubai doctors online, often late at night, in their own time zone.	Visual: three profiles side by side
12–20s	They check three things: is your specialty obvious, do your videos answer their exact concern, and do your reviews mention it?	OST: "Specialty · Their concern · Reviews"
20–27s	Then they message. If the reply comes eight hours later, they've already booked someone else.	OST: "Reply in 8 hours = lost"
27–32s	Your first consultation with them happens online, and often while you sleep.	OST: "First consult = online"

CTA: Dubai doctors: how many of your inquiries come from outside the UAE? Rough guess below.

SCRIPT 6B · BOFU · 75 WORDS · ~29 S

The doctor vs. influencer test

Core message: Your profile shouldn't make you look famous. It should make patients trust you.
Why it's specialist, not generic: A 3-part audit with exact things to count and check (name field, 6 of the last 9 posts, highlights), not vague advice.

TIME	SAY	ON SCREEN / VISUAL
0–3s	Open your Instagram. Ten seconds. Doctor's page, or influencer's page?	OST: “Doctor or influencer?”
3–10s	Test one: does your name field say your specialty and city? ‘Dr. Name, Dermatologist, Dubai.’	OST: “1. Specialty + city in name” ✔/✖
10–18s	Test two: count your last nine posts. At least six should answer a patient's problem. Not a trend, not a sound.	OST: “2. 6 of 9 = patient problems” ✔/✖
18–24s	Test three: is there a ‘Meet the doctor’ and an FAQ in your highlights?	OST: “3. Doctor + FAQ highlights” ✔/✖
24–30s	Two misses means your page is working against you. Patients book trust, not fame.	OST: “Trust > fame”

CTA: How many did you pass: 0, 1, 2 or 3? Comment your score.

SCRIPT 7B · TOFU · 73 WORDS · ~28 S

200K views and zero patients? Check this one screen.

Core message: Views only matter if they come from people who can actually visit your clinic.
Why it's specialist, not generic: Gives an exact screen to check (audience top cities in Insights) instead of 'views don't matter'. A real diagnostic.

TIME	SAY	ON SCREEN / VISUAL
0–3s	Your Reel got 200,000 views and no new patients. Check one screen.	OST: “200K views · 0 patients”
3–11s	Go to Insights, then audience, then top cities. If your city isn't in the top three, those views can't walk into your clinic.	Visual: Insights → Audience → Top cities (demo account)
11–17s	That's why viral isn't the goal. Local is.	OST: “Viral ✖ Local ✔”
17–27s	Three fixes: name your treatment and city in the hook, give one clear next step, and reply to every DM within minutes.	OST: “Treatment + city · One step · Fast reply”
27–31s	Views are attention. Patients are the result.	OST: “Views = attention. Patients = result.”

CTA: Check your top cities now. Is your city in the top three? Yes or no.

SCRIPT CLINIC 1 · TOFU · 90 WORDS · ~35 S

Multi-doctor clinic? Your page shouldn't only be about the owner

Core message: The owner's brand builds authority. A brand built around every doctor builds the practice.
Why it's specialist, not generic: Shows exactly how: one lane per doctor, Instagram Collab posts so each doctor builds their own profile too, and the 'which doctor do I see?' DM answered before it's asked.

TIME	SAY	ON SCREEN / VISUAL
0–3s	Running a clinic with three or more doctors? If your page is only about you, you're leaving bookings on the table.	OST: “Multi-doctor mistake”
3–10s	Patients message one question all the time: which doctor should I see for this?	OST: “‘Which doctor do I see?’”
10–19s	Answer it before they ask. Give every doctor a lane: one for pigmentation, one for injectables, one for implants. Each one answers their patients' questions on camera.	OST: “One doctor = one lane”
19–25s	Post them as Collab posts, so it shows on the clinic page and the doctor's own profile.	OST: “Collab posts: clinic + doctor”
25–31s	Your personal brand builds authority. Your team's brand builds the practice.	OST: “Authority + practice”

CTA: How many doctors are in your clinic, and how many appear in your content?

SCRIPT CLINIC 2 · BOFU · 87 WORDS · ~33 S

The 4-line bio rebuild I do for every cosmetic dentist

Core message: Patients decide in seconds whether your profile is worth a message.
Why it's specialist, not generic: Exact line-by-line bio and name-field rewrite, plus the outcome-first rule for cosmetic dentistry, shown live.

> ⚠ **Note**
> If you show a real dentist's profile, get written permission first, or blur it.

TIME	SAY	ON SCREEN / VISUAL
0–3s	Every cosmetic dentist I work with gets this exact bio rebuild first.	OST: “The 4-line bio rebuild” · Visual: profile (with permission, or blurred)
3–10s	Name field: ‘Dr. Name, Cosmetic Dentist, City’, so local patients find you in search.	OST: “Name field = search”
10–22s	Line one: what you do and where. Line two: the result, ‘natural smiles, planned around your face’. Line three: proof, years or cases. Line four: one link to book.	OST: lines appear as you say them
22–30s	Then in the feed, lead with the finished smile, then show how you got there. Result first, process second.	OST: “Result first. Process second.”
30–34s	Followers are numbers. Patients who trust you before they call are the goal.	OST: “Trust before the call”

CTA: Want me to rebuild yours? Comment BIO and I'll review it.

SCRIPT CLINIC 3 · MOFU · 86 WORDS · ~33 S

Day 1 of building a clinic brand: I don't film anything

Core message: Brand first, content second.
Why it's specialist, not generic: Concrete method: one positioning question, and flagship treatments chosen from the clinic's last 3 months of bookings, not opinion.

TIME	SAY	ON SCREEN / VISUAL
0–3s	On day one of building a clinic brand, I don't film a single video.	OST: “Day 1: I film NOTHING.”
3–10s	First, I ask the owner one question: what do patients get here that they can't get down the street?	OST: “1. What can't they get elsewhere?”
10–19s	Second, I pull the last three months of bookings. The two treatments bringing the most revenue become the flagships.	OST: “2. Flagships from real bookings”
19–25s	Third, I fix the profile: clinic name, specialty and city in the name field. One colour palette, one font.	OST: “3. Profile rebuild”
25–30s	Film before you know who you are, and you'll look like every clinic in town.	OST: “Brand first. Content second.”

CTA: Clinic owners: what's the one thing patients get at your clinic that they can't get anywhere else?

SCRIPT 4 · MOFU · 74 WORDS · ~28 S

Two dermatologists, same content: one booked for weeks

Core message: The content did its job. The patients leaked out after it.
Why it's specialist, not generic: Five precise leak points, plus a 2-minute exercise to find your biggest one.

TIME	SAY	ON SCREEN / VISUAL
0–4s	Two dermatologists. Same evidence-based content. One is booked for weeks, the other is empty.	OST: “Same content. Different calendars.”
4–8s	The difference isn't the content. It's what happens after.	OST: “It's what happens after”
8–20s	Unanswered DMs. Price questions answered the next day. No follow-up after ‘I'll think about it’. And happy patients never asked for a review.	OST: “Slow DMs · Late prices · No follow-up · No reviews”
20–31s	Try this: take your last Reel. Write down views, profile visits, DMs and bookings. Find the biggest drop between two numbers, and fix that before you film again.	OST: “Views → Visits → DMs → Bookings”

CTA: Send this to a doctor friend who posts a lot and wonders why the calendar's empty.

SCRIPT 5 · MOFU · 71 WORDS · ~27 S

One personal message beats ten promotions

Core message: A discount tells patients you need them. A personal message tells them you remember them.
Why it's specialist, not generic: Gives the exact message and a 4-part formula a clinic can send today.

TIME	SAY	ON SCREEN / VISUAL
0–3s	Stop sending patients '20% off this month'. Send this instead.	OST: "Generic ✖ Personal ✔"
3–14s	'Hi Sara, it's been about six months since your filler. Most patients like a top-up around now. Want me to check availability next week?'	Visual: message highlighted word by word
14–21s	Her name. What she had. Why now. One easy question.	OST: "Name · Treatment · Why now · One question"
21–27s	Your system fills in the details, sent only to patients who agreed to hear from you.	Visual: CRM auto-fill
27–31s	Discounts say you need them. This says you remember them.	OST: "Remembered > discounted"

CTA: Save the formula and send one message today.

SCRIPT 6 · TOFU · 64 WORDS · ~25 S

As a doctor, here's where I'd use AI in a clinic, and where I never would

Core message: Automate the admin. Never automate clinical judgment.
Why it's specialist, not generic: A clear green / amber / red line drawn by a doctor who builds AI systems: credible to doctors, not generic tech talk.

TIME	SAY	ON SCREEN / VISUAL
0–3s	I build AI for clinics. As a doctor, here's where I'd use it, and where I never would.	OST: "AI: 🟢 🟠 🔴"
3–14s	Green: after-hours replies that say they're AI, appointment reminders, follow-ups, review requests sent to every patient.	OST: "🟢 Replies · Reminders · Follow-ups · Reviews"
14–22s	Amber: drafting notes or patient instructions, only if a clinician checks every one.	OST: "🟠 Drafts, clinician checked"
22–28s	Red: diagnosis or treatment plans. That stays with the doctor. Always.	OST: "🔴 Clinical judgment"
28–32s	Automate the admin. Never the judgment.	OST: "Automate admin. Keep judgment human."

CTA: Which one surprised you? Tell me below.

SCRIPT 7 · BOFU · 79 WORDS · ~30 S

The 7 systems behind a $30k+/month clinic

Core message: Visibility brings inquiries. Systems turn them into appointments.
Why it's specialist, not generic: Seven named systems, each with a precise standard (same day, one place, weekly), so it's a checklist, not theory.

TIME	SAY	ON SCREEN / VISUAL
0–3s	A clinic doing $30k or more a month runs on seven systems. Count how many you have.	OST: "7 systems. Count yours."
3–10s	One: every inquiry captured, from calls, WhatsApp, Instagram and Google.	OST: "1. Capture"
10–15s	Two: booking in one minute, not three phone calls.	OST: "2. Easy booking"
15–20s	Three: same-day follow-up for everyone who didn't book.	OST: "3. Same-day follow-up"
20–26s	Four: review requests to every patient. Five: recalls when patients are due back.	OST: "4. Reviews · 5. Recalls"
26–33s	Six: one CRM, not five apps. Seven: inquiries, bookings and no-shows reviewed every week.	OST: "6. One CRM · 7. Weekly numbers"
33–37s	Content brings the inquiry. Systems bring the patient.	OST: "Content → inquiry. Systems → patient."

CTA: How many of the seven do you have? Comment your number.